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Obsessive-Compulsive Disorder

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Continuing Education Activity

Obsessive-Compulsive Disorder (OCD) is a prevalent psychiatric disorder affecting 1% to 3% of the global population, characterized by intrusive thoughts, known as obsessions, and repetitive actions, or compulsions. These symptoms affect patients not only by consuming a significant portion of their time but also by causing marked distress and functional impairment. The complex etiology of OCD involves cognitive, genetic, and neural factors, making the condition's diagnosis challenging and necessitating the exclusion of other psychiatric conditions that present similarly. Further complicating matters, OCD frequently coexists with other psychiatric disorders, requiring comprehensive identification and treatment for optimal clinical outcomes. While the combination of medication and psychotherapy is generally effective, emerging evidence supports using neuromodulation techniques (eg, deep brain stimulation and transcranial magnetic stimulation) for treatment-resistant cases.

This activity reviews the epidemiology, etiology, clinical presentation, and management of OCD, providing healthcare professionals with the knowledge and tools to improve patient care for this complex and prevalent condition. Emphasizing the importance of prompt recognition for timely diagnosis and intervention, the activity presents evaluation strategies and offers guidelines for differentiating OCD from similar psychiatric disorders. Varying etiologies, clinical presentations, and prognostic factors pertinent to OCD are also reviewed. By participating in this transformative activity, healthcare professionals will augment their clinical expertise and enhance their collaborative capacities, significantly improving patient care in OCD management.

Objectives:

- Identify the criteria utilized to diagnose a patient with obsessive-compulsive disorder.
- Implement evidence-based treatment plans for patients with obsessive-compulsive disorder.
- Assess the diverse etiological factors contributing to obsessive-compulsive disorder.
- Implement interprofessional team strategies for improving care coordination and communication to improve outcomes for patients with obsessive-compulsive disorder.

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Introduction

Obsessive-compulsive disorder (OCD) is a disabling condition estimated to affect 1% to 3% of individuals throughout their lifetime.[1][2] This psychiatric disorder is characterized by obsessions and compulsions, which consume a significant amount of time and lead to notable distress and impairment. Obsessions refer to intrusive and repetitive thoughts, urges, or mental images that are challenging to control. These thoughts often lack a clear purpose and are accompanied by distress.[3] Compulsions involve repetitive actions or mental events that individuals with OCD feel compelled to perform to alleviate the distress caused by the obsessions or to prevent a feared consequence from occurring.[3] Additionally, individuals with OCD may also engage in avoidance behaviors of obsession-triggering situations.[2]

OCD is a heterogeneous condition that arises from a complex interplay of genetic and environmental risk factors.[4] Most adults are distressed by the ego-dystonic nature of their obsessions and are aware that their compulsive behaviors are abnormally excessive. Children often have difficulty describing their obsessions. In OCD patients, common obsessions and their associated compulsive behaviors include fear of contamination leading to excessive cleaning, fear of harm linked to repetitive checking of security measures, intrusive, aggressive, or sexual thoughts paired with mental rituals, and a focus on symmetry accompanied by ordering or counting.[5][6] Though hoarding behaviors are usually specific to hoarding disorder, they can occur in OCD to prevent perceived harm. These behavior sets are consistently observed worldwide, suggesting a degree of commonality in OCD symptom dimensions. OCD can also present with rarer symptoms, including scrupulosity, obsessive jealousy, and musical obsessions.[7][8][3]

The understanding of OCD has evolved significantly over time. Historically framed in religious terms as a moral failing or demonic possession, OCD was first medically described by Esquirol. Freud subsequently characterized the condition using the term *obsessive neurosis*, positing that OCD originated with a regression in the anal phase of psychosexual development.[9] In the third edition of the Diagnostic and Statistical Manual (DSM-III), OCD was grouped with phobias under a single diagnosis. Later, the DSM-IV classified the condition as an anxiety disorder. The DSM-5 has reclassified OCD into the category "Obsessive-Compulsive and Related Disorders," alongside conditions like hoarding and body dysmorphia. This reclassification acknowledges shared characteristics, such as phenomenology, comorbidity, and underlying neurobiological factors.[10] WHO lists OCD as 1 of the 10 most disabling conditions caused by financial loss and decreased quality of life.[11] In The Diagnostic and Statistical Manual of Mental Disorders fifth edition Text Revision (DSM-5 TR), which was published by the American Psychiatric Association (APA) in 2022, OCD sits under the category of *obsessive-compulsive and related disorders* where the following subcategories were placed:[3]

- OCD
- Body dysmorphic disorder (BDD)
- Hoarding disorder
- Trichotillomania
- Excoriation (ie, skin-picking) disorder
- Substance or medication-induced obsessive-compulsive and related disorder
- Obsessive-compulsive and related disorder as a result of another medical condition
- Other specified obsessive-compulsive and related disorder
- Unspecified obsessive-compulsive and related disorder

The diagnosis of OCD is based on clinical assessment determining whether the DSM-5 TR criteria are met, which specify that either obsessions or compulsions must be present, the behaviors must be time-consuming, taking ≥ 1 hour per day, and significantly disrupting daily life.[3] (Refer to the **History and Physical Examination** section for more information on the diagnostic criteria for OCD).

Etiology

The etiology of OCD is complex, encompassing diverse factors, including cognitive, genetic, molecular, environmental, and neural elements. Evidence from twin studies points to a significant genetic predisposition with an estimated heritability quotient of approximately 48%.[12][13] However, this estimate is reduced to 35% when maternal effects (eg, prenatal exposure to stress or infection) are considered.[14] However, despite significant efforts through candidate gene association studies, reproducible genetic markers for OCD remain elusive. Many of these studies have focused on neurotransmitter pathways involving serotonin, dopamine, and glutamate without definitive results.[12] Nevertheless, the gene SLC1A1, responsible for encoding the neuronal glutamate transporter EAAT3, has surfaced as a potentially significant candidate.[15][12]

Recent studies point towards OCD being fundamentally a network-based disorder.[16] The cortico-striato-thalamo-cortical (CSTC) loop has become a pivotal framework for understanding its pathophysiology.[16] The CSTC loop

involves a series of interconnected circuits that allow the prefrontal cortex to communicate with subcortical structures (eg, the striatum and the thalamus). This loop has 2 distinct pathways: the direct and the indirect. The direct pathway facilitates behavior initiation, whereas the indirect pathway inhibits or modulates these behaviors.[17] In OCD, hyperactivity in the direct pathway relative to the indirect pathway has been observed. This creates an imbalance that may cause repetitive, intrusive thoughts and compulsive actions.[13][18] Recent neuroimaging studies indicate heightened connectivity and activation within the CSTC loop in OCD.[13] Furthermore, OCD may occur with other neurological conditions that impact the CSTC circuitry, including Parkinson disease, Sydenham chorea, traumatic brain injury, Tourette syndrome, Huntington disease, and epilepsy.[19][20]

Early findings of the effectiveness of clomipramine, which has robust serotonin reuptake inhibition observed during treatment, emphasized the role of serotonin in the pathogenesis of OCD.[13] However, this serotonin-centric model has faced scrutiny because other serotonin-modulating agents like buspirone and ondansetron have not proven effective in OCD.[21][22] Emerging studies point towards the glutaminergic system in the onset and progression of OCD.[13] Pharmacological agents like riluzole and troriluzole, which affect glutaminergic neurotransmission, have shown preliminary benefits.[23][24] The efficacy of antipsychotic drugs when used for augmentation in OCD suggests dopamine's role in OCD pathology.[25][13] Imaging studies corroborate this by demonstrating increased dopamine concentrations in the basal ganglia of patients with OCD. Moreover, dopamine agonists can induce OCD-like behaviors in both animals and humans. Interestingly, enhancing cortical dopamine has also shown promise in alleviating OCD symptoms.[26] However, the findings are preliminary and lack direct evidence.

The autoimmune etiology provides another intriguing dimension to OCD, particularly in Pediatric Autoimmune Neuropsychiatric Disorders Associated with Streptococcal Infections (PANDAS) and Pediatric Acute-onset Neuropsychiatric Syndrome (PANS).[27] PANDAS is regarded as a subset of PANS. Unlike OCD, the onset of PANDAS and PANS is often sudden and severe, accompanied by additional symptoms, including handwriting deterioration, emotional lability, and episodic disease courses. These conditions are mediated by autoimmune responses triggered by infection, inflammatory reactions, or other toxins and have parallels with other autoimmune neuropsychiatric disorders (eg, Sydenham chorea).[27] Preliminary research indicates that striatal cholinergic interneurons may be the cellular targets of these autoimmune responses.[28]

Cognitive and learning-based models of OCD posit that maladaptive beliefs fuel obsessional anxiety, leading to compulsive behaviors aimed at mitigating such anxiety.[25] Several cognitive impairments have been identified as contributing to obsessional anxiety, including:

- Heightened responsibility
- Overemphasis on thoughts
- Controlling thoughts
- Threat overestimation
- Perfectionism
- Intolerance of uncertainty

Empirical evidence supports the effectiveness of these models; exposure to obsession-linked stimuli increases anxiety while engaging in compulsive rituals lowers it.[25] Additionally, the learning-based model argues that the underlying learning mechanisms are not inherently pathological. This aligns with the observation that nonclinical populations also experience obsessive-like thoughts without distress. These models have been instrumental in developing psychological therapies for OCD, substantiating their theoretical and practical utility.[29][25]

Epidemiology

OCD is a leading cause of psychiatric morbidity worldwide, affecting 1% to 3% of the population. Frequently, OCD is associated with comorbid psychiatric conditions.[3][13][25][30] OCD often has onset early in life and generally has a chronic course. The most common demographic range affected is from ages 18 to 29 years.[3] Interestingly, nearly a quarter of males display symptoms before age 10, whereas the disorder usually emerges during adolescence for females.[3] Additionally, the peripartum and postpartum phases are marked as periods of increased risk for women,

with OCD incidence during these times exceeding that in nonpregnant females.[31] Women are also about 1.6 times as likely as men to be affected by the disorder.[32]

A striking 90% of individuals with OCD meet the criteria for at least one additional psychiatric disorder, with anxiety disorders, mood disorders, impulse-control disorders, and substance use disorders being the most prevalent comorbid conditions.[3] Despite the disorder's significant impact, OCD frequently remains both underdiagnosed and undertreated, with only a small fraction of patients receiving appropriate medical care.[3][33]

Pathophysiology

While OCD is generally considered to stem from a mix of several etiological factors, some instances can be linked explicitly to neurological causes involving the basal ganglia.[12] Detailed evidence supports this from case reports associated with conditions such as Sydenham's chorea and ischemic events. These conditions result in disruptions to basal ganglia regions like the globus pallidus and caudate, leading to obsessive-compulsive behaviors.[13] In research conducted by the Enhancing Neuro Imaging Genetics through Meta-Analysis (ENIGMA) consortium, no marked structural brain differences were discerned between patients with OCD and healthy controls unless factoring in medication status.[34] Postmortem studies, albeit limited, have revealed abnormalities in the orbitofrontal cortex of patients with OCD.[35] Specifically, one such study found evidence for lower excitatory synaptic gene expression in the orbitofrontal cortex of subjects (N=8) when compared with unaffected controls.[35] These studies, however, have limitations, including small sample sizes.

History and Physical

The diagnosis of OCD is based on clinical assessment determining whether the DSM-5 TR criteria are met, which specify that either obsessions or compulsions must be present, the behaviors must be time-consuming, taking ≥ 1 hour per day, and significantly disrupting daily life.[3]

Obsessive-Compulsive Disorder Diagnostic Criteria

In DSM-5 TR, the diagnosis of OCD was based on the following criteria.

- The presence of obsessions, compulsions, or both which meet the following definitions:[3]
 - **Obsessions**
 - Recurrent and persistent thoughts, urges, or images that are experienced at some time during the disturbance are intrusive and unwanted and, in most individuals, cause marked anxiety or distress.
 - The individual attempts to suppress such thoughts, urges, or images with some other thought or action (ie, by replacing them with a compulsion).
 - **Compulsions**
 - Repetitive behaviors or mental acts that the person feels driven to perform in response to an obsession or according to rules that must be applied rigidly.
 - The behaviors or mental acts aim at reducing anxiety or distress or preventing some dreaded situation; however, these behaviors or mental actions do not connect realistically with what they are designed to or are excessive.
- The obsessions are time-consuming or cause clinically significant distress or impairment in social, occupational, or other critical functional areas.
- Obsessive-compulsive symptoms do not arise from the physiological effects of a substance (eg, a drug of abuse, a medication) or another medical condition.
- The symptoms of another mental disorder do not better explain the disturbance. Differential diagnoses should be considered before an OCD diagnosis is made. (Refer to the **Differential Diagnoses** section for more information on conditions with clinical features similar to OCD.) These symptoms should not be attributable to

other psychiatric or medical disorders. The belief that obsessions lead to compulsions is common; however, the relationship between the conditions is not always straightforward. Theoretically, obsessions and compulsions can occur independently of each other. Patients with OCD typically seek outpatient care and often possess insight into their condition, although exceptions exist in children or severe cases. These individuals usually experience distress due to the ego-dystonic nature of their symptoms.

Clinical Assessment

A thorough medical history and mental status examination are vital in diagnosing the condition, differentiating OCD from other disorders, gauging prognosis, and devising a treatment plan. During the assessment, determining whether the patient is grappling with obsessions, compulsions, or both is critical. Symptoms should also be categorized into the following specific dimensions.

- **Contamination:** cleanliness obsessions and cleaning compulsions
- **Harmful thoughts:** fears of causing harm and compulsive checking
- **Forbidden thoughts:** aggressive, sexual, or religious obsessions with corresponding mental rituals; often suggest a worse prognosis.^[3]
- **Symmetry:** compulsions (eg, repeating, ordering, and counting) ^{[3][25]}

Safety assessments are also conducted to look for immediate risks to the patient or others. Screening for comorbid conditions, including depression, bipolar disorder, and other anxiety disorders, should be performed, and the patient's past psychiatric and general medical history should be reviewed. All medications, supplements, and known allergies or sensitivities should be obtained also. Furthermore, clinicians should gather pertinent information on the patient's psychosocial background, including familial relationships, stressors, and educational history. A family history focusing on OCD and other psychiatric conditions is also obtained to provide a holistic view of the patient's health.

Additionally, OCP may have specific nuances in its presentation, such as with postpartum OCD, with which clinicians should be familiar. For instance, some new mothers may experience distressing, obsessive thoughts about harming their baby and may be reluctant to disclose these for fear of judgment or consequences. In such sensitive cases, building rapport and ensuring a confidential and nonjudgmental setting for disclosure becomes paramount. However, the clinician must vigilantly assess the safety of the patient and the child. An individualized treatment plan for each patient should be created using a comprehensive approach.

Mental Status Examination

The mental status examination (MSE) for patients with OCD may differ depending upon the severity of symptoms, specific manifestations of the disorder, and any coexisting conditions. The following are some MSE findings that are common during the clinical assessment.

- **Appearance and behavior:** Patients generally present as well-groomed but exhibit visible anxiety. Manifestations of OCD, such as frequent hand-washing, checking behaviors, or rearrangement of objects, are often observed during the interview and serve as a diagnostic indicator.
- **Psychomotor activity:** Individuals are observed engaging in repetitive actions (eg, tapping, checking, or constantly washing their hands). These behaviors are compulsively performed, even when causing visible distress to the patient.
- **Speech:** While articulation is generally coherent and goal-oriented, intrusive thoughts may periodically disrupt the flow of speech. Depending on the individual, these thoughts may or may not be vocalized during the examination.
- **Mood and affect:** Patients usually report feelings of anxiety or distress, and their emotional expression (ie, affect) appears consistent with these reported experiences, often displaying heightened signs of stress.

- **Thought content:** Obsessive thoughts differ among patients but commonly revolve around themes like contamination, harm to oneself or others, a quest for symmetry, or distressing sexual or religious beliefs. Compulsions to counteract these obsessions are frequently reported.
- **Thought process:** Generally linear and coherent, intrusive, obsessive thoughts intermittently interrupt the thought process. Patients usually acknowledge these thoughts as irrational yet find themselves compelled to respond with corresponding compulsions.
- **Perceptual abnormalities:** Unlike some other psychiatric disorders, hallucinations or illusions are seldom observed in OCD patients.
- **Cognition:** Patients typically remain alert and oriented to time, place, and person. Overall cognitive function is usually preserved, although sometimes compromised by the pervasive nature of the obsessive thoughts.
- **Insight and judgment:** A significant feature in most OCD cases is the preservation of insight. Patients often recognize the irrational nature of their obsessions and compulsions but report feeling powerless to control them. Judgment may specifically falter when resisting the urge to perform compulsive behaviors.

Evaluation

Screening for the correct symptoms of OCD is essential. A common tool is the short OCD screener. At 6 questions long and a sensitivity of 97%, this screening modality is a simple and effective way to identify patients with symptoms of OCD.[36] However, the most widely accepted tool to screen for OCD is the Yale-Brown Obsessive-Compulsive Scale (Y-BOCS).[37] The Y-BOCS rates on a scale from 0 to 40, with 40 indicating the most severe symptomatology.[38] This scale requires the patient to rank the following based on severity:[38]

- Time occupied by obsessive thoughts and compulsions
- Interference of obsessive thoughts
- Distress of obsessive thoughts
- Resistance against obsessions
- Degree of control over obsessive thoughts
- Time occupied by compulsive behavior
- Interference of compulsive behavior
- Distress associated with compulsive behavior
- Resistance against compulsive behavior
- Degree of control over compulsive behaviors

Treatment / Management

Psychotherapy

Cognitive-behavioral therapy (CBT) is an evidence-based psychotherapeutic intervention for OCD.[39][40] Within the framework of CBT, exposure and response prevention (ERP) emerges as the most empirically substantiated behavioral technique. ERP entails subjecting patients to stimuli that provoke anxiety while guiding them to abstain from subsequent compulsive behaviors.[41] Various delivery modalities, including individual and group settings, as well as in-person and internet-based formats, have proven effective in treating OCD.[41]

A key determinant of therapeutic efficacy is the patient's adherence to at-home assignments, most notably those involving home-based ERP exercises. CBT serves as a first-line treatment for OCD, especially when this technique aligns with the patient's treatment preferences, access to qualified clinicians is available, and no compelling comorbid conditions necessitate pharmacotherapy. Although meta-analyses suggest that CBT often outperforms pharmacological interventions, such conclusions should be drawn cautiously, taking into account variables such as

patient selection criteria and baseline severity of OCD symptoms.[3] Emerging research indicates that intensive CBT protocols, often condensed into a brief period and occasionally administered in an inpatient context, hold promise for both initial and advanced treatment of severe OCD cases.[42]

Pharmacotherapy

Selective serotonin reuptake inhibitors (SSRIs) are recommended as the first-line medications due to their proven efficacy, safety, and tolerability.[43] Higher dosages than those used for other anxiety disorders or major depressive disorder are typically prescribed, improving effectiveness but increasing the risk of adverse effects such as gastrointestinal and sexual complications.[44] Therefore, careful assessment of side effects is critical for individual dosage optimization. Uniform effect sizes of commonly used SSRIs have been observed in systematic reviews, yet each SSRI has its adverse effect profile.[45].

Selection criteria of SSRIs for OCD include prior treatment response, adverse event potential, drug interactions, coexisting medical conditions, and cost and availability of the drug.[3] Treatment guidelines recommend an 8- to 12-week SSRI trial to determine efficacy. However, recent meta-analyses show significant symptom improvement within the first 2 weeks of SSRI treatment.[46][47]. An open-label fluoxetine study indicated that early symptom reduction within 4 weeks predicted 12-week treatment success.[48] Maintenance treatment is generally advised for at least 12 to 24 months postremission, but longer durations may be needed due to relapse risk upon medication discontinuation.[49]

Clomipramine, a tricyclic antidepressant (TCA), was the first drug to show efficacy in treating OCD. Meta-analyses suggest its higher efficacy compared to SSRIs, but this finding requires caution as early clomipramine studies included fewer treatment-resistant patients. Direct comparisons indicate equivalent efficacy between clomipramine and SSRIs. SSRIs are preferred for long-term treatment due to their superior safety profiles and tolerability than other antidepressants.[3]

Approximately half of patients with OCD fail to respond to first-line treatments.[50] Factors predicting poor response include higher symptom severity, marked functional impairments, specific obsession and compulsion subtypes (eg, sexual, religious, and hoarding), limited insight, high comorbidity rates, and lack of adherence to treatment protocols.[3] Combinatorial strategies involving CBT and SSRIs can be effective for those demonstrating poor response.[3] However, CBT is frequently constrained by availability or patient intolerance to exposure techniques.

Alternative approaches include switching SSRIs, using suprathreshold doses, or switching to serotonin-noradrenaline reuptake inhibitors.[3] SSRIs may also be augmented with antipsychotics, tricyclic antidepressants (eg, clomipramine), or glutamatergic agents.[51][52][3] The combination of fluoxetine and clomipramine outperforms fluoxetine with an antipsychotic. However, the fluoxetine and clomipramine combination carries the risk of elevated blood levels of both drugs, leading to potentially severe adverse events (eg, seizures, cardiac arrhythmias, and serotonergic syndrome).[53] Augmentation with antipsychotics, especially risperidone and aripiprazole, shows some evidence of efficacy; however, only one-third of SSRI-resistant patients experience clinically meaningful improvements, warranting meticulous risk-benefit evaluations.[51] Glutamatergic system modulators (eg, memantine, N-acetylcysteine, lamotrigine, topiramate, riluzole, troloxole, and ketamine) have been explored as augmentation therapies and have shown some promise.[3] Among these, N-acetylcysteine has the most substantial amount of evidence.[54]

Neuromodulation

In addition to the established treatment options such as Exposure and Response Prevention (ERP), Serotonin Reuptake Inhibitors (SRIs), and antipsychotic augmentation of SRIs, there exists a limited array of other evidence-based alternatives. Nonetheless, several treatment modalities have shown promise for cases refractory to standard interventions.

- **Transcranial magnetic stimulation:** Studies employing repetitive transcranial magnetic stimulation (rTMS) targeted at various brain regions (eg, the dorsolateral prefrontal cortex [dlPFC], dorsomedial prefrontal cortex [dmPFC], and orbitofrontal cortex [OFC]) have yielded mixed outcomes. However, these studies generally support the idea of cortico-striatal hyperactivity as an underlying factor in OCD.[55] Deep transcranial magnetic stimulation (dTMS), which utilizes an H-shaped coil, can reach depths of 3 to 5 cm, targeting midline

structures such as the medial prefrontal cortex (mPFC) and anterior cingulate cortex (ACC). These areas are believed to be hyperactive in OCD.[56] Significantly, dTMS has gained US Food and Drug Administration (FDA) approval for the treatment of OCD.[57]

- **Stereotactic ablation:** The introduction of the stereotaxic frame during the last century significantly improved the accuracy of lesion-based treatments for persistent psychiatric disorders. Dorsal anterior cingulotomy and anterior capsulotomy are the most commonly employed ablation methods. The former disrupts the cingulum bundle to sever communication among essential limbic regions, while the latter isolates the anterior limb of the internal capsule to disconnect the OFC and dACC.[58]. Both interventions are designed to regulate hyperactive CSTC circuits, which are believed to underlie OCD.[59] Cingulotomy has shown 41%, whereas capsulotomy had a 54% effectiveness rate.[58]
- **Deep brain stimulation:** Deep brain stimulation (DBS) is a reversible, adjustable treatment for intractable OCD with response rates between 40% and 70%, involving the neurosurgical implantation of an electrode that can activate neighboring neural circuitry.[60][13] DBS targets the anterior limb of the internal capsule and the ventral striatum (VS). Regulatory endorsements exist, but adoption is limited by cost and required expertise. [13] Electrode placement remains under debate, focusing on deep gray matter structures (eg, VS or nucleus accumbens [NAc], anterior limb of the internal capsule, and subthalamic nucleus [STN]) or white matter pathways connecting the prefrontal cortex to the thalamus.[61] Emerging research challenges the notion of high-frequency DBS as “functional ablation,” pointing to complex therapeutic mechanisms.[13] DBS modulates activity in brain regions linked to OCD, and the degree of modulation correlates with symptom improvement. Different DBS targets may yield specific benefits; ventral capsule (VC) and VS targeting improve comorbid depression, while STN targeting improves cognitive flexibility.[62] Future DBS targeting may be individualized based on clinical or neurobiological measures.[13]

Differential Diagnosis

OCD has symptoms similar to several other psychiatric conditions. Differentiating OCD from these conditions is essential for accurate diagnosis and effective treatment planning, as part of the criteria for OCD diagnosis involves ensuring that the symptoms of another mental disorder do not better explain the disturbance. However, OCD may also occur along with other psychiatric disorders, which can complicate clinical diagnosis. The following are some commonly considered differential diagnoses and the features that can separate from OCD:[3]

- **Generalized Anxiety Disorders:** OCD involves irrational or odd obsessions, distinct from real-life worries found in generalized anxiety disorder. Compulsions are typically present in OCD but not in anxiety disorders.
- **Specific Phobia:** Unlike OCD, fears in specific phobia are circumscribed to particular objects or situations and don't involve rituals or compulsions.
- **Social Anxiety Disorder:** Fears with this condition are related to social interactions, and avoidant behaviors are aimed at reducing social fears rather than neutralizing obsessions.
- **Major Depressive Disorder:** Ruminative thoughts in major depressive disorder (MDD) are mood-congruent and not linked to compulsive behaviors, unlike the intrusive obsessions in OCD.
- **Body Dysmorphic Disorder:** This condition involves obsessions and compulsions related only to physical appearance.
- **Trichotillomania:** Compulsive hair-pulling without the presence of obsessions differentiates trichotillomania from OCD.
- **Hoarding Disorder:** Difficulty in discarding possessions characterizes hoarding disorder; if hoarding is driven by OCD-like obsessions, an OCD diagnosis is given instead.
- **Eating Disorders:** Unlike OCD, the focus of obsessions and compulsions in disorders like anorexia nervosa is on weight and food. Ritualized eating behaviors are associated with eating disorders.

- **Tic Disorders:** Tics and stereotyped movements are generally less complex than compulsions and are not aimed at neutralizing obsessions. A dual diagnosis may be warranted for overlapping symptoms.
- **Psychotic Disorders:** Although some OCD patients may have poor insight or delusional beliefs, they do not exhibit other psychotic symptoms like hallucinations.
- **Obsessive-compulsive personality disorder:** OCD is characterized by intrusive, distressing obsessions and compulsions aimed at alleviating this distress, with individuals often recognizing their symptoms as excessive. In contrast, obsessive-compulsive personality disorder (OCPD) involves a chronic pattern of perfectionism and rigid control, without the presence of obsessions or compulsions, and is often perceived by the individual as rational and desirable.

Prognosis

OCD is a chronic condition characterized by fluctuating periods of symptom exacerbation and remission. Due to this disorder, daily functioning is significantly impaired. Furthermore, OCD is associated with an elevated risk of mortality.[13][63] Despite the utilization of CBT and SSRIs, a substantial proportion of patients remain unresponsive. Specifically, between 25% and 40% of patients do not experience symptom alleviation when treated with either CBT or SSRIs.[39][13] Furthermore, only a minority achieve full remission, and approximately half of successfully treated patients continue to manifest residual symptoms.

OCD associated with hoarding symptoms generally results in a more unfavorable prognosis. Clinical data from DSM-IV field trials involving 431 patients revealed that the fear of harm was the most commonly reported obsessive symptom. A significant association between OCD and suicidal tendencies has been confirmed, with contributory factors including coexisting anxiety and depression, as well as a history of suicide attempts.[64] Additionally, the association between OCD and suicidal tendencies remains significant even when controlling for depressive symptoms or mood instability.[65]

Complications

OCD is included in the top 10 disabling disorders by the WHO. Patients with OCD tend to avoid situations that make them uncomfortable, which may lead to decreased social interactions and a poor quality of life. Most who struggle with OCD go undetected for years. If OCD goes untreated, the pattern is harder to break as structural changes to the brain take place.[66][67] Duration of untreated OCD is associated with worse clinical outcomes.[68] Early intervention is vital.

Consultations

Utilizing cognitive behavioral therapy (CBT) with a focus on exposure and response prevention is the cornerstone of nonpharmacological treatment for OCD. Therefore, seeking consultation with a highly skilled and experienced therapist in administering this specialized form of therapy is crucial. Medical consultations may vary depending on the severity and specific nature of the compulsions exhibited. For instance, if a patient engages in excessive hand-washing, dermatological issues like dermatitis may arise, necessitating consultation with a dermatologist. Comprehensive treatment should address the patient holistically, encompassing psychological symptoms and any resulting medical conditions. Coordination of care with other healthcare clinicians (eg, pediatrics or family medicine) is also vital, especially for monitoring potential side effects of medications, including weight gain and tics.

Deterrence and Patient Education

In OCD, the patient's insight is not lacking. Only 2% to 4% lack insight into their OCD.[69] However, most people do not seek treatment until the disorder has become severely advanced. As most symptoms present during adolescence, clinicians should inform and educate appropriate individuals, including parents, fellow medical personnel, and those in the school systems, about this disorder.

Enhancing Healthcare Team Outcomes

Managing OCD requires an integrated, interprofessional healthcare team to offer patient-focused care, optimize treatment outcomes, ensure patient safety, and maximize team efficiency. The team comprises primary care physicians, psychiatrists, clinical psychologists, occupational therapists, pharmacists, and social workers, each with a distinct but collaborative role. Effective communication among team members is critical for optimal patient care.

Primary care physicians usually serve as the initial point of contact and are responsible for quickly identifying OCD symptoms and referring patients to specialized care. Psychiatrists oversee diagnosis and pharmacotherapy, while clinical psychologists conduct specialized psychometric tests to either confirm or rule out the OCD diagnosis, in addition to providing cognitive behavioral therapy. Clear, open communication among team members enables rapid intervention. Occupational therapists assess the patient's daily functioning, and social workers facilitate access to community resources. Pharmacists play a crucial role in managing medication interactions and ensuring medication adherence, particularly in instances of polypharmacy.

Ethical considerations like obtaining informed consent are paramount, as is aligning care with autonomy, beneficence, and nonmaleficence principles. Shared decision-making is emphasized, placing patient preferences at the center of all treatment decisions. Ongoing education and professional development keep the team updated on best practices in OCD treatment. This interprofessional approach to OCD management prioritizes comprehensive, safe, and high-quality patient care.

Review Questions

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